

Controlled Substance Diversion Prevention

Policy Number	POL-PH-035	Owner / Department	Pharmacy / Compliance
Effective Date	2023-09-15	Version	2.4
Last Revised	2026-02-14	Next Review	2028-02-14
Approved By	Pharmacy & Therapeutics Committee	Applies To	All staff with access to controlled substances, including prescribers, pharmacists, nurses, and pharmacy technicians

1. Purpose

To prevent, detect, and respond to the diversion of controlled substances through access controls, reconciliation, witnessed waste, and surveillance, in compliance with the federal Controlled Substances Act and DEA regulations at 21 CFR Part 1300 and following.

2. Definitions

- **Diversion** — the transfer of a controlled substance from a lawful to an unlawful channel of distribution or use.
- **Chain of custody** — the documented, unbroken trail of accountability for a controlled substance from receipt to administration, waste, or return.
- **Witnessed waste** — disposal of an unused controlled-substance portion verified in real time by a second authorized witness.
- **ADC** — automated dispensing cabinet providing controlled, auditable access to medications.

3. Policy and Procedure

Controlled substances are secured, inventoried, and reconciled per DEA requirements. Access is limited to authorized staff with unique credentials; biometric or individual login is required at every ADC transaction. Discrepancies are investigated and resolved before the end of shift.

1. Access controlled substances only under your own unique credential for a specific patient and order.
2. Reconcile counts at each handoff and resolve any discrepancy before leaving the unit.
3. Waste any unused portion immediately with a real-time authorized witness and document both names.
4. Return unused or recovered doses to Pharmacy through the approved return process; never pocket or discard unwitnessed.
5. Run and review ADC transaction and anomaly reports on the required cadence.

IMPORTANT — Suspected diversion or a controlled-substance count discrepancy that cannot be reconciled must be reported to the Pharmacy Diversion Officer and the supervisor immediately. Do not confront the suspected individual; preserve all records and secure the medication and equipment involved.

4. Red Flags and Required Response

Indicator	Response
Unreconciled count discrepancy	Investigate before end of shift; escalate if unresolved.
Excessive waste or unwitnessed waste	Review transactions; interview; refer to Diversion Officer.
Removals without a matching order	Audit ADC logs; compare to MAR; secure records.
Tampered packaging or substituted product	Quarantine product; notify Pharmacy and Security immediately.

5. Severity & Escalation

Classify diversion events on the POL-RM-013 scale. Confirmed diversion carries mandatory external reporting to the DEA (via DEA Form 106 for significant loss or theft) and the state board as required.

Severity	Example	Notify / timeframe
SEV-1	Confirmed diversion causing patient harm or substituted/diluted patient doses.	Diversion Officer, Nursing Supervisor & Risk Mgmt within 1 hour.
SEV-2	Confirmed diversion without patient harm; significant unresolved loss.	Diversion Officer & Compliance within 4 hours.
SEV-3 / Near Miss	Reconciled discrepancy or policy lapse with no diversion confirmed.	Event report within 24 hours.

6. Documentation Requirements

- Drug, strength, quantity, and the complete chain of custody from removal to administration/waste.
- Witness name and time for every waste event.
- Count discrepancies, investigation steps, and resolution.
- Any external report filed (DEA Form 106, state board) with date and reference number.

7. References

- Controlled Substances Act; DEA regulations at 21 CFR Part 1300 et seq. (security, recordkeeping, disposal).
- DEA Form 106 — Report of Theft or Loss of Controlled Substances.
- CMS Conditions of Participation, 42 CFR 482.25 (pharmaceutical services).
- Related: POL-RM-013 (severity & reporting); POL-PS-003 (medication-error reporting).